SACTCHECK · TREATMENT PASSPORT

Adjuvant dabrafenib and trametinib after removal of BRAF-mutated stage III melanoma

A record of what you experience

One review period per page. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal CTCAE grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Take dabrafenib twice daily, around 12 hours apart, and trametinib once daily, every day unless the team instructs a hold. The adjuvant course is planned for 12 months, unless stopped earlier for recurrence, side effects or another clinical reason. A four-week review is not a drug holiday or one of 12 chemotherapy cycles.

These pages are reusable recording sheets; the prescribed course length is stated above. Ask for additional sheets as needed.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Your own hospital / contact instructions:

https://sactcheck.com/patient/00415/adjuvant/

Full patient information, sources and visual timeline. No identifiers are encoded.

Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending

My diagnosis, plan and medicines

Complete these pages with your team. Leave unknown or inapplicable items blank. This record supports the consent discussion and your written prescription.

Diagnosis and overall plan

Original melanoma site / pathology date

Stage / Breslow thickness / ulceration

Lymph-node or other disease sites

BRAF result / date / other relevant tumour tests

Advanced or adjuvant intent / earlier surgery or RT

Medicines and safety

Anticancer medicines / actual dose / route / start date

Treatment received, missed or held / restart instruction

All other medicines / allergies / important previous reactions

Pregnancy-prevention or immune-alert plan / continuing precautions

Oncology / out-of-hours / pharmacy contacts

NCCN Guidelines for Patients: Melanoma, 2026 - supplied education reference; original fields adapted to the Irish NCCP regimen context. No NCCN form or artwork reproduced.. Your Irish treating team confirms the diagnosis, prescription and follow-up plan.

My checks, symptoms and support

Record what each test is checking and when the result will be discussed. Your team explains which examinations or scans are needed for your diagnosis and treatment.

Test and purpose	Date / booking contact	Result discussion	Agreed next step

Record bloods, skin/lesion examinations, scans and specialist checks separately. Tumour-marker and scan intervals are individually selected; do not infer them from the cycle calendar.

Symptoms, skin and support

New symptoms / onset / change / action taken

Lesion or wound / pain / eating / weight / daily function

Skin self-check plan / optional dated photograph record kept privately

Sun protection / practical, emotional or family support needs

Questions, response review and the outcomes that matter to me

My urgent contact plan / what my carer should do

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Review period: _____ Start date: _____ Next review: _____

Record the actual date, treatment received, any hold, important symptom and the next check. A review visit is not automatically a medicine dose or a completed cycle.

Adjuvant course: monthly review record, planned 12 months · copy this sheet for another review period of the same phase.

Dabrafenib · Oral BRAF inhibitor

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Fever / chills	☐ No fever or chills	☐ New chills / feeling unwell: call	☐ Fever: use the written urgent plan now	☐ Fever with marked weakness: urgent assessment	☐ Collapse / confusion / breathing difficulty: 112/999
Skin	☐ No change	☐ New small change: report	☐ Persistent rash / itch: contact team	☐ Painful, spreading or blistering rash: urgent review	☐ Severe skin reaction with acute illness: emergency help
Fatigue	☐ Usual energy	☐ Tiredness improves with rest	☐ Rest does not help; usual tasks affected: call	☐ Self-care difficult: urgent review	☐ Collapse: 112/999

Trametinib · Oral MEK inhibitor

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Eyes	☐ Usual vision	☐ New eye symptom: contact team	☐ Persistent eye pain / blur: urgent advice	☐ Sudden vision loss: immediate urgent assessment	☐ Vision change with stroke signs: 112/999
Breathing	☐ Usual breathing	☐ New cough / change: call	☐ Worsening breathlessness: urgent advice	☐ Breathless at rest: urgent assessment	☐ Severe breathing difficulty: 112/999
Swelling	☐ None	☐ New swelling: report	☐ Increasing swelling: contact team	☐ One painful swollen leg / breathlessness: urgent review	☐ Severe chest pain / breathing difficulty: 112/999
Bowels	☐ Usual pattern	☐ New change: report	☐ Persistent diarrhoea / constipation: call	☐ Blood, severe pain or dehydration: urgent review	☐ Collapse or severe sudden illness: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.